



ISPD Asia-Pacific Chapter Newsletter, August 2026

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Prepared by Chia-Te Liao and Talerngsak Kanjanabuch

News from the ISPD

Message from ISPD-APC Chapter Convenor

One of the greatest privileges of serving our Chapter is seeing colleagues growth and then watching that growth return to their own communities. This year, six colleagues will represent our region at **ISPD 2026 in Cape Town**: Dorjsuren Munkhjavkhlan (Mongolia), Isabella Panelo (Philippines), Jing Guo (China), Naveen Kumar Medi (India), Theerachai Thammathiwat (Thailand), and Vikas Sharma (India). Each receives a travel grant of **USD 1,300** together with complimentary registration. We received 15 excellent applications, making the selection especially competitive. I am particularly pleased that one award goes to an allied health professional. PD depends on multidisciplinary teams, yet nurses and allied health professionals often have the fewest opportunities for international training. That pathway must remain open.

The same philosophy underpins our Fellowship Programme. Congratulations to **Dr Marjoa Humaira Mekhola** and **Dr Sarif Mahammad Salauddin**, both from Bangladesh, who have been awarded the 2025 Second Round Fellowships. Supported by up to **USD 3,000**, they will train at approved PD Centres of Excellence before returning home to strengthen local kidney care. At the same time, applications for the **2026 Second Round Fellowships** are now open. Up to Two awards of **USD 4,000** will support 2-3 months of training within our region. I strongly encourage renal nurses, PD coordinators, dietitians, pharmacists, technicians, and young nephrologists to apply before **31 December 2026**. Investing in people remains the most sustainable investment we can make.

That investment felt even more urgent after the **Nephrology Society of Fiji Conference 2026**, where teams from 6 Pacific nations each had 8 minutes to describe kidney care in their countries. The numbers are sobering. Fiji's kidney failure incidence is 4-5x that of Australia; its dialysis population grew from 229 to 337 in <3 years; 93% of patients start dialysis with a central venous catheter; and 3-year survival is only 34% among those aged 45-64 years. Tonga still has no dialysis service. Samoa supports >230 HD patients but has no PD, and 1 in 3 dies within a year of dialysis initiation. In Kiribati, donated dialysis machines remain idle because consumables have expired. In Vanuatu, 3 nurses, only 1 formally trained in dialysis, care for 12 patients from a building declared unsafe after a magnitude 7.4 earthquake.

These are not isolated challenges, but recurring patterns across the Pacific. Kidney disease is increasing fastest where access to kidney care remains most limited. Yet PD, arguably the modality best suited to dispersed island communities, is often unavailable, leaving many patients with little realistic access to long-term kidney replacement therapy.

As we look ahead to Busan 2027, and celebrate the 25th Anniversary of the ISPD Asia-Pacific Chapter, I hope we continue to invest not only in science, but also in people, partnerships, and places that have too often remained at the margins. The future of PD in our region will be shaped not only by the centres with the greatest resources, but also by how effectively we support those with the greatest need.



Professor
Talerngsak
Kanjanabuch, M.D.

Thank you, as always, for your friendship, collaboration, and unwavering commitment to advancing peritoneal dialysis throughout the Asia-Pacific region.

Warmest regards,

Professor Talerngsak Kanjanabuch, M.D.

ISPD-APC Chapter Convenor and ISPD Council Member

ISPD Asia-Pacific Chapter Scholarship Application

This is a scholarship to support **up to 3 months** training in clinical PD for doctors and nurses from Asia-Pacific region. The purpose is to promote PD awareness, knowledge, and expertise by visiting a center of excellence.

Applicants must be **ISPD members**.

- For medical applicants, candidates who have completed their nephrology training and with some working experience as a junior consultant would be preferred.
- For applicants of other healthcare professionals, candidates who have a working experience of at least 5 years would be preferred.

Each application must include the following documents: (1) A completed application form along with (2) a curriculum vitae, (3) a recommendation letter from the director of the nephrology unit or department, (4) a letter of acceptance from the institution to be visited; and (5) a motivation letter.

A letter of support from the administrator of the applicant's institution is advisable.

The next deadline is **31 December 2026**.

Details and application procedures can be found under the Regional Chapters – Asia-Pacific Chapter, at the ISPD website. [Asia-Pacific Chapter - International Society for Peritoneal Dialysis](#)

Renew your membership!

Individual memberships

- For Trainees and Fellows: A very discounted membership for USD \$60 per year (after providing proof of status).
- For Doctors: Full membership for one year costs USD \$175, but there are great discounts for 2- and 3-year membership packages. Moreover, in 2026 we introduced a new discounted membership fee for all Low Income Countries (LIC) and Lower-Middle Income Countries (LMIC) at USD \$60 per year!
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 - An Associate Nurse/AHP option is also available for \$15 USD per year (with limited benefits).
- Retired Professionals can enjoy a discounted membership for \$80 USD per year (after providing proof of status).

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- Society Membership: Your national society can contact the ISPD and negotiate a discounted price for groups of members.
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Visit <https://ispd.org/memberships/> to join the ISPD or renew your membership.

Membership benefits of the ISPD include:

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- ISPD Newsletter sent every two months and ISPD APC Newsletter sent every 4 months
- Direct access to all ISPD Educational materials: ISPD Guidelines, Webinars, Journal Clubs, Case Discussions...

- Special registration fees at ISPD events, including the ISPD APC Meeting in September 2027 in Busan (Korea) and the ISPD 2026 Congress in Cape Town.
- Opportunity to apply for ISPD Scholarships and Grants

Upcoming Meetings

ISPD 2026 Congress

29 September - 3 October 2026

Cape Town International Convention Centre (CTICC), Cape Town, South Africa

You can review the program [HERE](#) and meet the speakers [HERE](#).

More information: [ISPD 2026 website](#)



ISPD APCM 2027

1 October - 3 October 2027

Busan, South Korea

More information coming soon



Research News from Asia-Pacific Region

When Choice Outran Capacity: The Hidden Cost of Dialysis Reform



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Key Reference: Phannajit J, Praditpornsilpa K, Tungsanga K, et al. Outcomes and trade-offs of Thailand's 2022 patient-choice dialysis policy reform. *Kidney Int Rep* 2026;11:106377.

Colleagues, Thailand's PD-First policy is not only a dialysis story. It is a lesson in what happens when a national policy touches the real working parts of a health system: money, workforce, logistics, incentives, infrastructure, and time.

Introduced in 2008 under the Universal Coverage Scheme, which covers roughly 70-75% of the Thai population, PD-First made peritoneal dialysis (PD) the default kidney replacement therapy. Hemodialysis (HD) was reserved for patients with clear contraindications. This was not a philosophical preference for one modality over another. It was a practical answer to a national constraint. Thailand needed to expand dialysis access, but HD units, trained staff, and capital investment were limited. PD offered a route the system could actually support: home-based care, lower infrastructure demand, and favorable cost-effectiveness [1].

The policy delivered. PD use increased more than tenfold, from about 2,760 patients to more than 25,000. PD reached 20-30% of prevalent dialysis patients and 60-70% of incident dialysis patients, while outcomes remained comparable with international benchmarks [2]. But this success was not luck. Thailand built the runway before takeoff. Training was expanded. Protocols were standardized. Supply chains were secured. Community-based care was strengthened [2]. The policy worked because the system had been prepared to carry it. That is the first lesson: **readiness must come before scale.**

PD-First also changed who could receive care. By embedding dialysis within a tax-funded universal coverage system, Thailand reduced financial barriers for patients who had previously been excluded from life-saving treatment [3]. Equity did not appear as a side effect. It was built into the design. Yet even strong systems accumulate strain. Patient numbers grew faster than the workforce. Nurse-to-patient ratios became stretched. Home dialysis increasingly depended on family caregivers [4]. At the national level, outcomes still looked stable. On the ground, the system was carrying more weight than it showed. This is the **Success Paradox**: a program can look robust from above while its foundations are weakening below.

The strain became visible after 2022, when Thailand moved from PD-First to unrestricted patient choice between PD and HD. The principle was right. Patients should have autonomy. But choice is not a form. It is not a slogan. It is a clinical promise. And every promise needs capacity behind it. The shift was immediate. Incident HD use rose from about 50% to more than 75%, making HD the dominant modality [5]. PD use among incident patients fell from about 50% to 14% [2].

HD chairs expanded. The rest of the system could not move as fast. Vascular access services lagged. More patients started dialysis without planned access. Risk increased at the most vulnerable point in the patient journey: treatment initiation [5]. The clinical signal came early. Ninety-day mortality among incident dialysis patients increased from about 9-13%, with the excess concentrated among HD patients [5]. This is the sharpest lesson from Thailand's transition: **choice without capacity is not empowerment. It is risk transferred to the patient.**

The financial signal was just as clear. Dialysis expenditure rose by nearly 74% and consumed a growing share of the national health [5]. When broader system costs were included, dialysis approached 12% of total UCS spending while serving less than 1% of beneficiaries. System-dynamics modeling suggested that, without correction, dialysis could consume 20-30% of the national health budget [2]. Health systems do not fail only at the bedside. They fail in queues, budgets, bottlenecks, incentives, and delayed decisions. A policy can be clinically well intentioned and still fiscally unsafe if growth outpaces design.

Thailand's strength was its response. In 2025, the country did not simply return to the old PD-First model. It recalibrated. The reform introduced structured predialysis education, preauthorization, and stronger governance [6]. Early signals suggest recovery. PD uptake among incident patients rebounded to 58%. Unplanned initiation declined. Vascular access improved. Ninety-day mortality fell to about 7%, not only returning to baseline but improving beyond it [2]. That may be the most important part of the story. High-performing systems are not systems that never stumble. They are systems that detect stress early, learn fast, and correct course before harm becomes routine.

Thailand's experience leaves five practical lessons.

- **Sequence before scale.** Workforce, infrastructure, financing, supply chains, and clinical pathways must be ready before policy expands demand.

- **Choice must be supported.** Autonomy matters, but unsupported choice can become unsafe care.
- **Watch the warning lights.** Unplanned dialysis initiation, catheter dependence, vascular access delays, and early mortality are not routine indicators. They are system alarms.
- **Incentives shape care.** Guidelines describe what should happen. Payment structures, provider behavior, and market forces often decide what actually happens.
- **Systems determine outcomes.** The central question is not PD versus HD. The central question is whether the system can deliver the chosen modality safely, equitably, and sustainably.

Thailand's PD-First experience is therefore not a narrow argument for one dialysis modality. It is a blueprint for reform under constraint. It shows that equitable dialysis expansion is possible when policy is matched by preparation. It also warns that even a strong system can falter when reform moves faster than readiness.

For countries facing the rising burden of kidney failure, the message is direct: build the system before expanding the promise. Sequence reform carefully. Monitor early. Correct quickly. Because in health care, success is not measured by the elegance of a policy. It is measured by whether patients live longer, safer, and better.

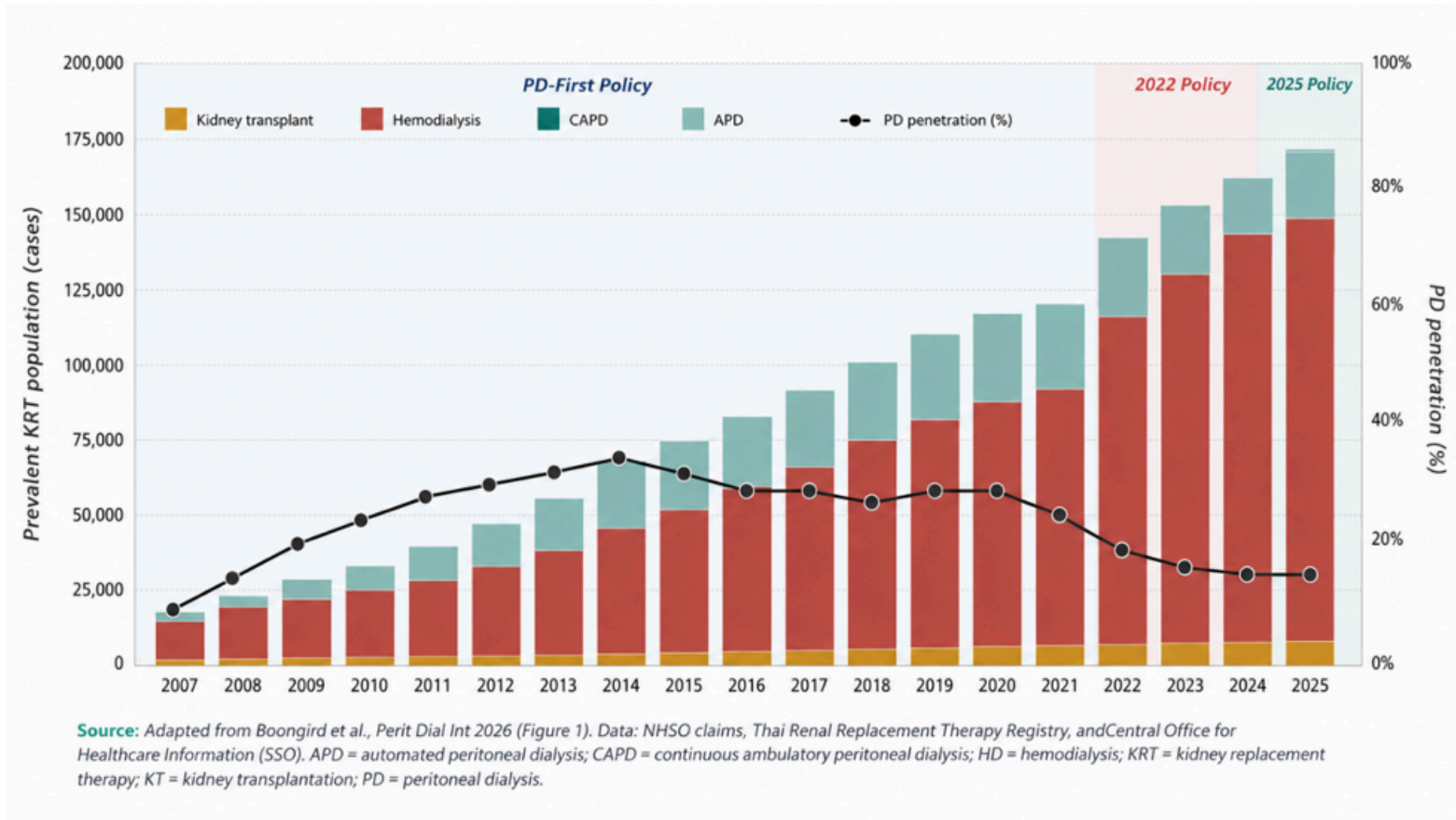


Figure 1. Evolution of Thailand's kidney replacement therapy (KRT) policy and modality mix (2007–2025).

National prevalent KRT population stratified by kidney transplantation, hemodialysis (HD), continuous ambulatory peritoneal dialysis (CAPD), and automated peritoneal dialysis (APD), with PD penetration (%) overlaid. The figure illustrates three distinct policy eras: the PD-First policy (2008–2021), the unrestricted "Free Choice" policy (2022–2024), and the governance-driven 2025 KRT reform. The marked decline in PD penetration following policy liberalization, despite continued growth in overall KRT utilization, provides the health-system context for the present study examining frontline implementation of the 2025 reform. Data were adapted from Ref #2.

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Taiwan's Peritoneal Dialysis Challenge: Outstanding Outcomes, Declining Utilization, and a Call for Policy Reform



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Why Excellent PD Outcomes Have Not Translated Into Wider PD Adoption in Taiwan

Peritoneal dialysis (PD) offers several well-established advantages over in-center hemodialysis, including better preservation of residual kidney function, greater patient autonomy, lower healthcare costs, and an early survival benefit in selected populations [1, 2]. Yet Taiwan faces a striking paradox. Despite achieving some of the best PD outcomes worldwide, its PD uptake has steadily declined over the past decade. The incidence of PD fell from 14.2% in 2007 to 9.5% in 2022, while prevalence declined from 9.6% in 2011 to 7.5% in 2022 [3, 4]. These trends highlight a growing disconnect between PD's clinical value and the healthcare system's capacity to support its wider adoption.

The reasons behind this decline are complex. Taiwan's dialysis population is aging and becoming increasingly complex, making modality selection more challenging. In addition, patients and families may not always receive timely or comprehensive education about dialysis options, and shared decision-making (SDM) has not been uniformly implemented. Historically, reimbursement structures have favored in-center hemodialysis, creating institutional incentives to default to facility-based care even when PD is an appropriate option [3]. Reversing this trend has therefore required not only clinical advocacy, but also deliberate policy reform.

The Taiwan NHI Peritoneal Dialysis Reimbursement Initiative: PROMOTE-PD

In July 2023, Taiwan's National Health Insurance Administration launched a national PD reimbursement initiative, hereafter referred to as PROMOTE-PD, in collaboration with the PD Working Committee of the Taiwan Society of Nephrology. Since its launch, approximately US\$13 million annually, equivalent to 1% of the annual dialysis budget, has been allocated for PD development [3].

PROMOTE-PD pursues two complementary goals: increasing PD use, with near-term targets of incidence above 10% and prevalence above 8%, while improving quality simultaneously [3]. Growth alone is not success. Expansion must be safe, sustainable, and patient-centered.

PROMOTE-PD operates at several levels. Financial incentives support new PD units and the reopening of previously closed units. A mentorship model links experienced medical centers with new or smaller units, strengthening training, clinical capacity, and referral pathways [3]. PROMOTE-PD also reimburses structured SDM for patients with stage 5 CKD and those within two years of dialysis initiation. Structured education can increase patients' willingness to consider self-care dialysis [5]. By valuing the decision-making process, it helps ensure that modality choice reflects patients' goals and circumstances rather than institutional preference.

Quality measures have been embedded from the outset. Participating institutions are assessed across multiple domains, including peritonitis, hospitalization, dialysis adequacy, and preservation of residual kidney function [3]. Telemedicine is also reimbursed, extending specialist support to patients in rural and remote areas [6].

Short-Term Results: Early Signs of Success

Results from the first six months suggest that PROMOTE-PD has already begun to reshape PD delivery in Taiwan. Six previously closed PD units resumed service, and 68 new units were established, including seven in district hospitals and 61 in community clinics. Crude PD prevalence rose from 7.5% to 7.9%, marking the reversal of a long-standing downward trend [3].

Quality performance was equally encouraging. Medical centers achieved 100% compliance with the quality framework, regional hospitals reached approximately 90%, and overall adherence exceeded 85% [3]. These early

results show that reimbursement reform can expand PD capacity without compromising quality, especially when financial incentives are aligned with accountability and professional support.

Looking Forward: The Taiwan 2026-2035 Home Dialysis White Paper

The continued implementation of PROMOTE-PD provides a strong operational foundation for the Taiwan 2026-2035 Home Dialysis White Paper, issued by the Ministry of Health and Welfare, the Taiwan Society of Nephrology, and the International Society for Peritoneal Dialysis. This White Paper serves as a ten-year roadmap for expanding both PD and home hemodialysis. Built on three guiding principles, namely person-centered care, healthy ageing, and sustainable healthcare systems, it outlines five strategic pillars and phased targets: PD prevalence of 8% by 2025, combined PD and home hemodialysis prevalence of 12% by 2029, and 18% by 2035, with an interim review planned for 2028 (Figure 1) [7].



Figure 1. Strategic framework and phased targets of the Taiwan 2026-2035 Home Dialysis White Paper, issued by the Ministry of Health and Welfare, Taiwan Society of Nephrology, and the International Society for Peritoneal Dialysis [7].

These goals will be advanced through five strategic pillars: payment reform, patient empowerment, coordinated care, innovative service delivery, and quality excellence [7]. Notably, PROMOTE-PD has already translated many of these priorities into practice through dedicated reimbursement, professional training, structured SDM, community-based service expansion, telemedicine, and outcome-based quality monitoring. As such, it serves not only as a response to declining PD uptake, but also as a key implementation platform for Taiwan's broader transition toward home dialysis.

Implications for the Asia-Pacific Region

Taiwan's experience offers important policy insights for countries seeking to expand home dialysis. International evidence suggests that financial incentives alone are rarely enough. Payment reform must be paired with professional training, patient empowerment, and strong delivery-system infrastructure [8]. PROMOTE-PD reflects this broader strategy by combining reimbursement reform with mentorship, shared decision-making, community-based service expansion, and quality accountability. Hong Kong's experience similarly highlights the importance of sustained policy commitment and strong professional infrastructure [9].

Equally important, quality must be built into expansion from the outset, and shared decision-making should be recognized as a reimbursable clinical intervention rather than an informal discussion. Longer-term follow-up will be needed to assess technique survival, cost-effectiveness, patient-reported outcomes, and the program's impact on older and frailer patients. Continued evaluation will remain essential as the initiative evolves.

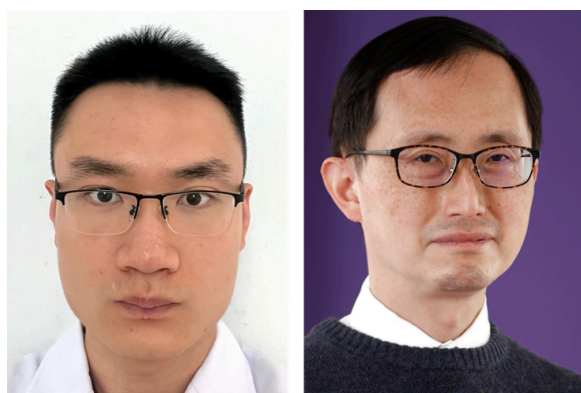
Conclusions

Since its launch in 2023, PROMOTE-PD has remained active and continued to support PD development nationwide, helping Taiwan reverse the decline in PD use while maintaining high standards of care. More importantly, it has repositioned home dialysis from a recommended treatment option to a long-term national policy priority. In doing so, Taiwan is redefining dialysis success not only in terms of survival, but also through autonomy, dignity, access, quality, and sustainability.

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High Body Roundness Index Correlated with Worse Outcomes in Peritoneal Dialysis Patients



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Key Reference: Xu L, Ng JK, Chan GC, et al. Body roundness Index on the clinical outcome of peritoneal dialysis patients: Analysis of two cohorts. *Perit Dial Int* 2026;8968608261446827.

Central obesity is a common complication with important clinical implications in patients undergoing peritoneal dialysis (PD) [1]. Nonetheless, current techniques that can assess visceral fat levels, such as CT and MRI, are too costly and impractical for routine clinical use. Meanwhile, the body roundness index is a novel anthropometric measure that requires only body height and waist circumference to estimate central obesity and visceral fat levels [2]. Unlike the traditional body mass index (BMI), which cannot distinguish between muscle and fat, BRI directly reflects the level of central adiposity. Therefore, in the general population, BRI serves as a robust predictor of metabolic syndrome, cardiovascular disease, and even all-cause mortality [3-5]. Since metabolic syndromes and cardiovascular events are common in PD patients [6], BRI is a promising non-invasive screening tool for risk-stratification in PD patients. Nonetheless, the clinical implications of BRI have not been thoroughly studied in PD patients.

Recently, we have published a paper investigating the role of baseline body roundness index (BRI), an anthropometric measurement to reflect visceral fat and central obesity, in two independent cohorts of PD patients [7]. The exploratory cohort contained 249 prevalent PD patients, and the validation cohort contained 162 incident PD patients. Baseline BRI was calculated as $364.2 - 365.5 \times (1 - [\text{waist} \times 2\pi^2] / [0.5 \times \text{height}]^2)$, while other clinical and demographical data were recorded as part of clinical care. In the exploratory cohort, the average BRI was 4.7, with more than one-third of patients having a BRI greater than 5. The multivariate Cox regression analysis

was performed to adjust for potential confounders, and it demonstrated that BRI was an independent predictor for all-cause mortality and technique failure. Each one-unit increase in BRI was associated with 34.6% higher risk of all-cause mortality (AHR = 1.346, $p = 0.005$) and a 27.9% higher risk of technique failure (AHR = 1.279, $p = 0.004$). Furthermore, we also found that a high BRI was strongly related to a higher hospitalization rate: patients with a BRI of 6 or higher had nearly three times as many hospital admissions per year (0.77 versus 0.25) and five times longer total hospital stays (4.62 days per year versus 0.91) compared to those patients with a BRI below or equal to 4 (**Figure 1**). On the contrary, baseline BRI was not associated with peritonitis-free survival ($p = 0.783$).

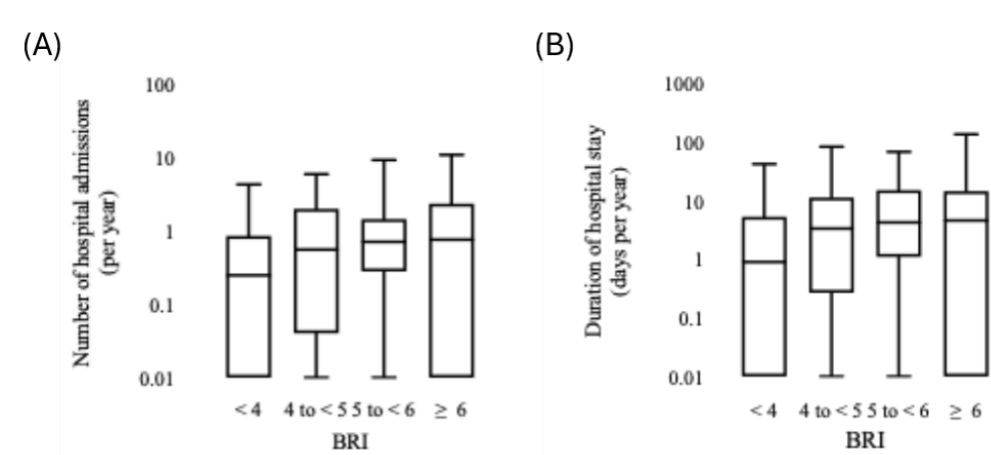


Figure 1. Box plots showing the relationships between different BRI groups and (A) the number of hospital admissions per year, and (B) the duration of hospital stays per year.

However, the validation cohort yielded some inconsistent results. In this group of incident PD patients, baseline BRI was associated with patient survival in the univariable analysis, but after adjusting for confounding clinical factors such as age and Charlson's comorbidity scores, BRI failed to remain as an independent predictor. Similarly, BRI was also not an independent predictor of remaining alive on PD in the validation cohort after adjusting for potential confounders. Interestingly, in this second cohort, BRI showed a significant association with peritonitis-free survival (AHR = 1.243, $p = 0.028$), which was not observed in the exploratory cohort. These inconsistent findings may be explained by the fact that the validation cohort had a smaller sample size than the exploratory cohort (162 vs 249), and the patients in the exploratory cohort were prevalent PD patients with long dialysis vintage (median 36 months), while patients in the validation cohort were all incident PD patients.

Several critical limitations of this study should be noted. For instance, the single-center design restricts generalizability to other populations or treatment settings. In addition, the exploratory cohort consisted of prevalent PD patients who had already survived a median of three years on dialysis, which may lead to survival bias that obscures the true relationship between BRI and survival outcomes (8). More critically, the inconsistency between the exploratory and validation cohorts lowered the certainty of our findings. Since the inconsistent findings are likely caused by the smaller sample size in the validation cohort as well as the longer dialysis vintage in the exploratory cohort, future studies should further validate our findings by increasing the sample size and investigating incident PD patients only.

Despite these limitations, our study offers a crucial insight to clinicians: while BMI showed no significant association with any survival outcomes in either cohort, BRI could independently predict mortality and hospitalization in the exploratory cohort. This finding strongly supports our hypothesis that central obesity is a more important risk factor than overall body weight in PD patients, and BRI could serve as an appropriate indicator of visceral fat.

In conclusion, BRI has significant prognostic value in PD patients in predicting survival, technique failure, and hospitalization, and a high BRI is associated with increased all-cause mortality and technique failure rates, as well as more hospital admissions. The inconsistent findings from the exploratory and validation cohorts, however, limited the certainty of our findings, suggesting that further research is required to validate our findings in larger PD populations. Nonetheless, given that BRI is easy to assess, we strongly recommend incorporating BRI assessments into routine clinical care and serving as a recommended guideline for risk stratification in PD patients.

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The associations between quality of life and a five-day per week peritoneal dialysis prescription: A pilot randomised controlled study



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Introduction

Peritoneal dialysis (PD) is typically performed daily; however, there is a trend for accommodating patients with residual kidney function to receive a less burdensome PD prescription, also known as incremental PD [1]. Examples of this prescription include fewer exchanges on continuous ambulatory PD, lower volumes or fewer cycles on automated PD, absence of a day dwell, as well as having dialysis-free days [2, 3]. Despite increasing adoption of the incremental PD prescription and endorsement by the International Society for Peritoneal Dialysis [4, 5], the evidence base supporting its use and any potential benefits are largely based on retrospective cohort studies [6, 7]. Furthermore, no prior studies have formally examined the effects of having two dialysis-free days a week on patient outcomes. While there may be benefits in reducing the treatment burden associated with daily PD, there remain the potential risks of inadequate dialysis, fluid overload and increased hospitalisation by prescribing incremental PD. This study aimed to determine the feasibility, impact on quality of life (QOL) and safety outcomes of a 5-day a week PD regimen.

Methods

This was a pilot, multi-centre, randomised controlled trial conducted in Melbourne, Australia. Incident PD patients with 24-hour urine volumes of >500 mL and creatinine clearance of >50 L/week/1.73m² were recruited and randomised 1:1 to receive 5-day a week PD or daily PD, for 12 months. Those treated with 5-day PD could have their PD volumes and exchange/cycles increased if clinically required within the constraint of 5-day dialysis, while the indications to increase to daily PD included refractory uraemic symptoms, hyperkalaemia > 6.5 mmol/L, volume overload or if residual creatinine clearance declined to <50 L/week/1.73m². The primary outcome was the duration of 5-day PD. Secondary outcomes included QOL measured by KDQOL-SF v1.3 survey, time on PD, peritonitis rate and PD-related hospitalisation.

Results

Twenty-four participants were recruited, with n=11 on 5-day PD and n=13 on daily PD. At baseline, there were no significant differences in age, sex, diabetes mellitus comorbidity, serum creatinine between the two groups. The type of PD (continuous ambulatory or automated), use of neutral pH solutions were also similar between groups. Weekly Kt/V was lower in the 5-day group (2.1 ± 0.3 vs. 2.5 ± 0.5), reflective of an incremental prescription. The

mean and median duration of 5-day PD were 8.4 ± 4.7 and 12 (IQR 5-12) months, respectively. Six participants (55%) continued 5-day PD for the entire follow-up period. Three 5-day PD participants (27%) incremented to daily PD due to refractory uraemic symptoms; two were within the first 3 months. These three participants also had the lowest weekly Kt/V at baseline. Over 12 months, the 5-day group saved >10,000 L of PD solution utilisation and on average, had 73 dialysis-free days per participant.

There were no differences in the physical, mental and kidney disease composites of QOL between the two groups at baseline or at the end of follow-up. When the components of the kidney disease composite were analysed, 'patient satisfaction' (94.4 ± 8.3 vs. 81.5 ± 13.0 , $P < 0.05$) and 'social support' (87.5 ± 11.8 vs. 63.0 ± 29.8 , $P < 0.05$) were higher at 12 months in the 5-day group compared with the daily PD group. Importantly, 'symptoms' were not different between the groups. Sleep quality was also captured by the KDQOL-SF survey which showed a significant deterioration in the daily PD group over 12 months (**Figure 1**). Sleep quality was higher in the 5-day group compared to the daily PD group at 12 months (60.6 ± 14.8 vs. 46.7 ± 7.1 , $P < 0.05$). Time on PD, rates of peritonitis and hospitalisation were not different between the two groups. Similarly, biochemical results such as serum potassium, phosphate and the peritoneal membrane solute transfer rate were comparable between groups.

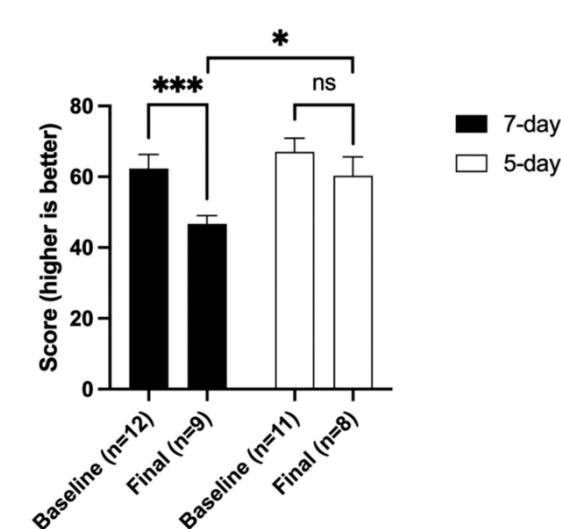


Figure 1. Sleep quality as expressed as mean $\pm$ SEM. Pair-wise comparisons demonstrated a significant reduction in sleep quality in the daily PD group ($P < 0.001$), whereas no change was seen in the 5-day group. At 12 months, sleep quality was higher in the 5-day group ($P < 0.05$).

Discussion

This pilot study provides insight to the feasibility and the potential benefits to QOL with a 5-day a week PD prescription in patients with preserved residual kidney function. Importantly, there were no obvious signals of harm such as increased hospitalisation or reduced time on PD. A 5-day PD regimen was able to be continued for a median of 12 months, which allows more time for patients to engage in life participation. This may explain the higher 'patient satisfaction' and 'social support' in the incremental PD group. In addition, having two dialysis-free days a week had tangible benefits in preserving sleep quality, reducing glucose exposure from PD solutions, and improving the environmental impact of PD. Due to the small sample size, the findings from this pilot study are hypothesis generating and require validation in a larger and heterogeneous cohort.

Conclusion

Five-days per week incremental PD may be a safe and feasible starting prescription in patients with preserved residual kidney function in the first year of dialysis. An international randomised controlled trial on incremental PD is underway and results are eagerly awaited.

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Randomized Clinical Trial on the Efficacy and Safety of Preoperative Enema in the Prevention of Peritoneal Dialysis Catheter Dysfunction



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The success of peritoneal dialysis (PD) mainly depends on a well-functioning peritoneal catheter [1]. Current guidelines from the International Society for Peritoneal Dialysis (ISPD) [2] and the Chinese Standard Operating Procedures for Peritoneal Dialysis [3] specifically recommend bowel management for constipated patients prior to catheter insertion. This recommendation primarily stems from two observational studies conducted in 1996 [4] and 2004 [5], which reported substantially higher rates of initial dialysis dysfunction in constipated patients compared to non-constipated counterparts (68% vs. 13%). However, several knowledge gaps persist. First, the reported prevalence of constipation in CKD/ESKD populations ranges modestly from 14.2% to 28.9% [6,7], leaving uncertainty about optimal management for the majority without baseline constipation. Second, existing evidence derives from small-scale observational studies lacking standardized protocols for bowel preparation. Third, no randomized controlled trials have evaluated the efficacy and safety of routine preoperative bowel preparation in contemporary PD practice.

Our preliminary single-center retrospective analysis [8] revealed a significant association between preoperative bowel preparation and reduced catheter dysfunction risk (HR 0.384, 95%CI 0.254-0.581, $p < 0.001$). However, retrospective designs inherently limit causal inference and may introduce selection bias. To address these limitations and provide higher-level evidence, we conducted this prospective randomized controlled trial evaluating the efficacy and safety of preoperative enema in preventing catheter dysfunction among PD patients.

We enrolled consecutive patients who needed catheter insertion for PD in our center. i) Patients had to be 18 years or older, ii) Maintenance peritoneal dialysis treatment is required for ESKD, iii) First-time open surgeon peritoneal dialysis catheterization patient, iv) Only straight double-cuff Tenckhoff dialysis catheters were used, v) Willing to participate in this study and sign informed consent form. Exclusion criteria were as follows i) known contraindication to PD or enema, ii) the patient refused enema, iii) expected survival time < 3 months and iv) cathartic agents such as senna, lactulose, or colonic dialysis and enema were used 3 days before PD surgery. Recruitment took place from Jan 2022 through Dec 2023. Patients underwent randomization and were assigned in a 1:1 ratio to either enema group or control group. The primary outcome measure of this study was the incidence of catheter dysfunction requiring intervention within 3 months. Catheter dysfunction refers to drainage failure, as defined by the inability to drain peritoneal dialysate effluent reliably within 45 minutes. Secondary outcomes included the incidence of catheter dysfunction requiring intervention in six months, catheter event free survival time, the incidence of peritoneal dialysis-associated peritonitis, and other catheter complications.

A total of 136 patients were randomly assigned to enema group or control group. One patient randomized to the control group was excluded from the final analysis because scrotal leakage on the first day after PD catheter insertion. Therefore, 67 patients in each group included in the per-protocol population, respectively. Baseline characteristics showed no significant intergroup differences. Of the included patients, 10 (14.7%) and 11 (16.2%) had constipation in the enema and control groups, respectively. And 16.9% of included patients had previous abdominal surgery before catheter insertion. The mean interval between enema administration and PD catheter insertion was 9.96 hours, ranging from 4 to 16 hours. No patients were lost to follow-up during the trial.

There were 35 (25.7%) episodes of catheter dysfunction requiring intervention at the end of the 3-month follow-

up. Patients randomized to the enema group had a significantly lower catheter dysfunction rate than the control group (14.7% vs. 36.8%, unadjusted RR 0.297 [95% CI 0.129-0.682], $p = 0.004$; adjusted RR 0.291 [95% CI 0.121-0.737], $p = 0.009$; **Table 1**). With respect to the primary end point of catheter dysfunction, the Kaplan-Meier event free survival probability for the two groups was shown in **Figure 1**. Log-rank test for the difference in catheter event-free survival curves indicated the superiority of enema ($p = 0.0015$).

Table 1. Clinical and Catheter Outcomes in Intention-to-Treat Population.

Variables	No./total (%)		Unadjusted		Adjusted [#]	
	Enema group	Control group	Risk ratio	<i>p</i> value	Risk ratio	<i>p</i> value
	(n = 68)	(n = 68)	(95% CI)		(95% CI)	
Primary outcome						
Catheter dysfunction requiring intervention at 3 months (%)	10 (14.7)	25 (36.8)	0.297 (0.129-0.682)	0.004	0.299 (0.121-0.737)	0.009
Secondary outcomes						
Catheter dysfunction requiring intervention at 6 months (%)	11 (16.2)	28 (41.2)	0.276 (0.123-0.617)	0.002	0.272 (0.114-0.648)	0.003
Catheter tip migration at 3 months (%)	10 (14.7)	18 (26.5)	0.479 (0.203-1.132)	0.094	0.489 (0.188-1.271)	0.142
Catheter tip migration at 6 months (%)	11 (16.2)	20 (29.4)	0.463 (0.202-1.062)	0.069	0.471 (0.190-1.172)	0.106
Catheter dysfunction requiring surgical intervention (%)	0	1 (1.5)	NA	0.997		
Safety and adverse events						
Peritoneal dialysis associated peritonitis (%)	1 (1.5)	3 (4.4)	0.323 (0.033-3.189)	0.334	0.309 (0.026-3.605)	0.349
Leakage (%)	10 (14.7)	12 (17.6)	0.805 (0.322-2.011)	0.642	0.776 (0.300-2.011)	0.602
Tunnel/exit infection (%)	2 (2.9)	2 (2.9)	1.000 (0.137-7.311)	1.0	2.486 (0.172-35.936)	0.504
Death (%)	1 (1.5)	1 (1.5)	1.000 (0.061-16.320)	1.0	1.959(0.081-47.427)	0.679
Patients with bleeding from incision	3 (4.4)	2 (2.9)	1.523(0.246-9.416)	0.651	0.682(0.046-10.055)	0.781
Bloody peritoneal dialysis effluent	1 (1.5)	0	NA	0.997		
PD Hydrothorax	2 (2.9)	1 (1.5)	2.030(0.180-23.933)	0.567	5.757(0.239-138.90)	0.281
Others	Pneumonia 1	Blood transfusion 2				

[#]Adjusted for age, sex, body mass index, history of abdominal operation, constipation, surgeon, serum potassium

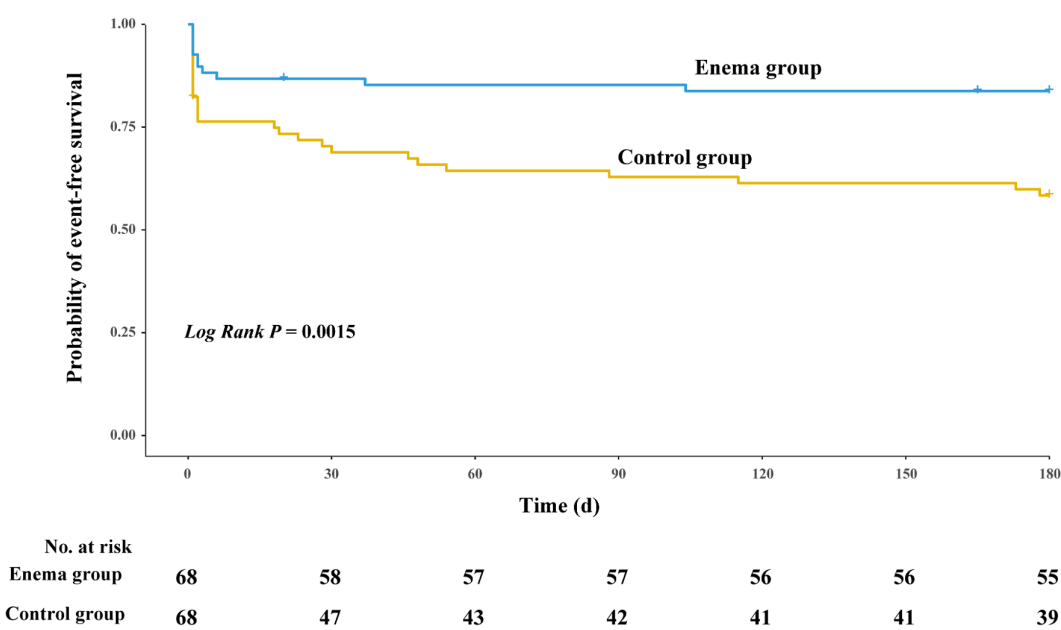


Figure 1. Kaplan–Meier survival curves for catheter event-free survival in intention-to-treat population.

After six months following, catheter dysfunction was more frequent in the control group: 28 of 68 patients (41.2%) compared with 11 of 68 patients in the enema group (16.2%, $p = 0.002$). Catheter tip migration was the most common cause of catheter dysfunction. As shown in Table 1, 10 of 68 (14.7%) patients receiving enema had catheter tip migration versus 18 of 68 (26.5%) in control group within 3 months. The slightly higher incidence of catheter tip migration in the control group, but the difference between the groups was nonsignificant. Catheter dysfunction required surgical intervention only in one patient in the control group. No statistical differences were

observed between the two groups in terms of leakage, peritoneal dialysis-associated peritonitis, and exit or tunnel infection.

Overall, the 6-month follow-up data analysis showed that preoperative enema significantly reduces early PD catheter dysfunction risk without increasing complication rates. These findings support incorporating bowel preparation into standard preoperative protocols for PD catheter placement.

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A New Validated Tool for Measuring Life Participation in Dialysis: An Update from the SONG-LP Study



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Key Reference: Burnett CT, Hughes A, Jaure A, et al. Validation of the Standardized Outcomes in Nephrology - Life Participation (SONG-LP) Instrument in People Receiving Dialysis. *Kidney Int Rep* 2026;11:106608.

I am Dr Cameron Burnett, an interventional nephrologist, PhD candidate based in Brisbane, Queensland and a father of three. I am pleased to share with the ISPD community that a measurement of life participation, the Standardised Outcomes in Nephrology-Life Participation (SONG-LP), has been validated in people receiving dialysis in our study published in Kidney International Reports involving 250 adults receiving dialysis (67 of whom were receiving peritoneal dialysis) from 13 countries [1].

As a background, the SONG initiative was established in 2014 to develop core outcome sets for trials across the spectrum of kidney disease, built on consensus among patients, caregivers and health professionals [2]. Life participation has been consistently identified as a core patient-reported outcome to be included in trials across kidney transplantation, peritoneal dialysis, early CKD, paediatric populations and glomerular disease. Life participation is defined as the ability to engage in meaningful activities such as work, family, social connection and leisure [3]. With this latest publication, the SONG-LP instrument has now been validated across the spectrum

of kidney disease, including in kidney transplant recipients³ and in people with chronic kidney disease not yet receiving kidney replacement therapy [4].

Owing to only recent recognition as a core outcome and a lack of specific, validated instruments to measure it, life participation has been inconsistently or indirectly captured in PD trials until now. This study addresses that gap. The SONG-LP scores correlated well with the PROMIS Ability to Participate in Social Roles and Activities scale (PROMIS-APS) and EQ-5D-5L, widely-used measures that relate to social functioning and independence (Spearman's rho = 0.76 and 0.62, respectively). The SONG-LP showed that its four individual questions scored similarly, measured the same underlying concept (Cronbach's α = 0.89) and scores remained stable when the same participants completed it again a week later (82% completed this follow-up). This suggests that the SONG-LP measure can capture life participation in a meaningful and reliable way. The instrument also performed consistently across people receiving PD, home HD and in-centre HD, supporting its use as a unifying tool of measurement across dialysis modalities, which will be valuable for researching life participation across the spectrum of kidney disease.

Several other important next steps are already underway. Firstly, the feasibility of using the SONG-LP measure will be determined at a clinical level, including how it might be incorporated into clinical workflows. A study is underway in Brisbane, Australia at an early adopter PD unit to begin addressing this question. Secondly, the necessary tools (such as a user manual) will be developed to support its routine use in clinical trials together with dissemination and support of key international stakeholders. Thirdly, determination of the minimally important clinical difference will allow us to better understand how life participation score changes should be interpreted at patient, clinical and research levels. Fourthly, the effectiveness of SONG-LP requires determination – i.e., does the use of the SONG-LP trigger changes in clinical management or facilitate patient-centred conversations in clinic that drive better patient-centred care? Lastly, translation and cross-cultural validation will be necessary to ensure relevance and appropriateness of SONG-LP use across a wide range of clinical settings. Already, a French translation of the SONG-LP has been completed [5], and a Spanish translation is currently underway.

This work represents an important step forward for person-centred care, both at the clinical level and within the research community, providing for the first time a brief, validated means of capturing what matters most to our patients with kidney disease.

Should your institute be interested in using the SONG-LP in a clinical or research setting, please do not hesitate to contact me directly at Cameron.burnett@health.qld.gov.au, or visit the SONG initiative website at <https://songinitiative.org/>.

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New Joint ISRNM/ISPD Guideline will be released soon!

The highly anticipated paper entitled ***Joint International Society of Renal Nutrition and Metabolism (ISRNM) and International Society for Peritoneal Dialysis (ISPD) Practice Recommendation on Nutrition Management in Peritoneal Dialysis*** will be jointly released by **J Renal Nutr** and **Perit Dial Int** by both societies with FREE online access to this document soon! In the coming **ISPD 2026 congress**, Professor Angela yee-moon Wang from Singapore and Miss Zarina Ebrahim, dietitian from South Africa will present this new joint Societies guideline to the attendees in the session **‘Transforming nutrition care in PD’** on 2 oct 2026.